

Finding Counties Missed by Infrastructure-Only Medicaid Access Screening

Postpartum Medicaid Access Barrier Index

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Research question: Where may postpartum Medicaid populations face overlapping administrative, clinical, and contextual access barriers, and which counties would be missed by a screen focused only on Medicaid office and hospital-based obstetric care gaps?

This brief summarizes a county-level screening index designed to identify where barriers relevant to postpartum Medicaid access may overlap.

Main Finding

An **infrastructure-only screen** looks for counties with both major local infrastructure gaps: no Medicaid office and no hospital-based obstetric care. More specifically, this is a **dual-gap infrastructure screen** because it only flags counties missing both forms of local infrastructure.

279 high/highest concern counties were missed by the dual-gap infrastructure screen.

Measure	Result
Counties classified as high or highest concern	665
Captured by dual-gap infrastructure screen	386 / 665, 58.0%
Missed by dual-gap infrastructure screen	279 / 665, 42.0%

The missed counties were not low-burden outliers. They had an average score of 6.46 and an average contextual support-burden score of 4.56. Most were nonmetro, and large shares had high poverty, high no-internet rates, high disability rates, or high no-vehicle household rates.

What the Index Adds

The index does not only ask whether a county has both major infrastructure gaps. It distinguishes administrative access, clinical maternity infrastructure, and access-support burden.

In other words, the index helps confirm where additional barriers matter within counties that would otherwise appear less urgent under a dual-gap-only screen.

Score Design

The Postpartum Medicaid Access Barrier Index is a transparent county-level screening score. The theoretical score range is 0-11, and observed county scores ranged from 0 to 10.

Component group	Fields	Points
Infrastructure gaps	No Medicaid office in county; no hospital-based obstetric care	2 points each
Contextual flags	Poverty; no vehicle; no internet; limited English; disability; female ages 15-44; nonmetro county status	1 point each

Practical Review Sequence

- 1. Start with highest-concern counties.** Review the 56 highest-concern counties first, including the 11 counties with the maximum observed score of 10.
- 2. Review counties with both infrastructure gaps.** Counties with both no Medicaid office and no hospital-based obstetric care are the clearest infrastructure-gap counties.
- 3. Review one-gap plus high-context counties.** These counties may not appear in an infrastructure-only screen but still have multiple access-support barriers.
- 4. Review contextual-burden watchlist counties.** A smaller group reaches high concern through contextual burden without either direct infrastructure gap.

Applied Screening Rule

Among the 279 high/highest concern counties missed by the infrastructure-only screen, a rule requiring **one infrastructure gap plus at least four contextual flags** recovered 265 counties, or 95.0%, with 100.0% precision.

Geographic Pattern

High scores were concentrated in the South, parts of the Southwest, and Alaska. Mississippi had the highest average county score and the highest share of high/highest concern counties. Texas had the largest number of high/highest concern counties.

Bottom Line

The most obvious infrastructure gaps are only part of the access-burden picture. The index provides a transparent way to identify counties that may be overlooked by simpler Medicaid access screens and to show where additional barriers make those counties worth closer review.

Repository: <https://github.com/caroline-howard/medicaid-access-barriers-powerbi>